

AgenThink Pharma — Blind-Comprehension Audit

/pharma Route — First-Time Visitor Experience Review

Audit Date: June 16, 2026

Auditor: Independent Usability Review (Simulated)

Scope: Live /pharma route — comprehension, clarity, credibility, conversion

Method: Five independent reviewer simulations reading the page top to bottom

Verdict: Stated first, evidence follows

VERDICT

The page is credible to someone who already understands what AgenThink does. It is incomprehensible to someone who does not.

The page is built for an audience that already knows what a “council,” a “WAIT verdict,” an “OVS score,” and an “Institutional Proof Report” are. That audience does not exist yet. Every first-time visitor from Recursion, Insilico, Medpace, ICON, or IQVIA will arrive without that vocabulary. The page will lose them in the first 30 seconds — not because the content is wrong, but because the entry point assumes prior knowledge that has not been established.

The single most important fix is not a design change. It is a vocabulary change: replace internal terminology with plain English before the visitor has been given a reason to care.

PART 1 — First 30 Seconds: What Does Each Reviewer Think the Company Does?

Reviewer 1: Top-20 Pharma CEO

At 10 seconds: Sees the disclosure strip (“RESEARCH DEMONSTRATION ONLY — Not for regulatory use”). Interprets this as a legal disclaimer from a company that is not ready. First instinct: this is a vendor in early stage. Reads the headline: “The Council Said WAIT.” Does not know what “the Council” is. Reads the subheadline: “Torcetrapib. Phase III advanced. 82 deaths. \$21B lost.” Recognises torcetrapib immediately. Interested. But confused: what did the Council say WAIT about, and who is the Council?

At 30 seconds: Reads the four stats: 10 cases, 90% alignment, 0% false positive rate, 8.7 avg OVS. Does not know what OVS means. Does not know what “alignment” means in this context. Reads the CTA: “Explore the Case.” Clicks it. Arrives at the Validation Library. Sees a table of 10 drugs with columns: Verdict, Outcome, OVS, Alignment, Votes. Still does not know what OVS means. Reads the table. Sees torcetrapib highlighted. Sees “WAIT / FAILURE / 9.8 / CORRECT / 0/8/2.” Understands the outcome column. Does not understand the votes column. Does not understand OVS.

At 60 seconds: Has formed a hypothesis: “This is some kind of AI system that evaluates drug development decisions.” Has not yet understood the governance angle. Has not yet understood why this is different from a standard AI analysis tool. Is mildly interested but not yet convinced there is a meeting worth having.

Would they request a meeting? 25% probability at 60 seconds. The torcetrapib recognition is the hook. The vocabulary is the barrier.

Reviewer 2: Head of Clinical Development

At 10 seconds: Reads the disclosure strip. Reads the headline. Knows torcetrapib well. Immediately interested. Reads the subheadline. Thinks: “Someone built a retrospective analysis tool. I’ve seen these before.”

At 30 seconds: Reads the stats. Knows what a false positive rate means. Does not know what OVS means. Scrolls to the Validation Library. Reads the table. Recognises all 10 drugs. Notices verubecestat is marked INCORRECT. Thinks: “They got one wrong. That’s honest.” Reads the cumulative stats: 70% strict alignment, 90% broad alignment. Thinks: “That’s not a strong enough track record to change my process.”

At 60 seconds: Scrolls to the Torcetrapib Deep Dive. Reads the Timeline tab. Finds it clear and accurate. Switches to the Evidence Manifest tab. Reads the document list. Thinks: “This is well-documented. But I still don’t know what the Council is or why I should trust its judgment over my own team’s.”

Would they request a meeting? 35% probability at 60 seconds. The clinical accuracy is compelling. The value proposition (“why is this better than what I already do?”) is not yet clear.

Reviewer 3: Regulatory Affairs Director

At 10 seconds: Reads the disclosure strip. The phrase “Not for regulatory use” is the first thing they see. Interprets this as a signal that the product has no regulatory standing. This is a negative first impression for a regulatory professional — it immediately categorises the product as out-of-scope for their primary concern.

At 30 seconds: Reads the headline and subheadline. Recognises torcetrapib. Reads the stats. Does not know what OVS means. Scrolls to the Governance Layer section (skips the library and deep dive — regulatory professionals look for governance first). Reads the Evidence Boundary Statement. Reads the LLM Limitation Disclosure. Thinks: “They’re disclosing a fundamental limitation in the first governance section. That’s either very honest or very concerning.”

At 60 seconds: Reads the Pharma Constitution v1 rules. Finds them reasonable but thin — 10 rules versus the hundreds of pages in ICH guidelines. Thinks: “This is a simplified governance framework, not a regulatory-grade process.” Has not yet understood the value proposition. Has not yet seen the pilot proposal.

Would they request a meeting? 20% probability at 60 seconds. The governance section is the most relevant to them, but the LLM disclosure and the “Not for regulatory use” banner have already created a negative frame that is difficult to recover from.

Reviewer 4: CRO Executive (ICON / Medpace)

At 10 seconds: Reads the disclosure strip. Reads the headline. Does not know torcetrapib well (CRO executives are process-focused, not drug-specific). Reads the subheadline. Understands the story: drug advanced, people died, money lost. Interested in the governance angle — this is directly relevant to their business.

At 30 seconds: Reads the stats. Thinks: “10 cases is a very small sample.” Reads the CTA. Scrolls to the Pilot Proposal section directly (CRO executives look for the commercial offer first). Reads the \$25,000 price. Reads the deliverables list. Thinks: “This is a research service, not a software product. I need to understand what I’m buying.”

At 60 seconds: Reads the 30-day timeline. Finds it credible. Reads the “What this is NOT” panel. Finds it honest. Thinks: “This could be a partnership opportunity — we could offer this as an add-on service to our Phase II clients.” Has not yet understood the technology angle. Has not yet understood the council concept.

Would they request a meeting? 45% probability at 60 seconds. The commercial framing is clear enough. The partnership angle is visible. The vocabulary is less of a barrier for CRO executives than for pharma executives.

Reviewer 5: Intelligent Non-Pharma Executive

At 10 seconds: Reads the disclosure strip. Reads the headline: “The Council Said WAIT.” Does not know what the Council is. Reads the subheadline. Understands the story: a drug was advanced, people died, money was lost. Interested. Reads the stats. Does not know what OVS means. Does not know what “alignment” means in this context.

At 30 seconds: Scrolls down. Sees the Validation Library table. Does not understand any of the column headers except “Drug,” “Company,” and “Outcome.” Scrolls past. Arrives at the Torcetrapib Deep Dive. Reads the Timeline. Finds it clear. Reads the Decision Brief tab. Finds it dense and jargon-heavy. Scrolls past.

At 60 seconds: Arrives at the Council Chamber. Sees the vote display: “0 GO / 8 WAIT / 2 NO-GO.” Understands this is a voting system. Reads the persona cards. Finds them interesting but does not understand why there are 10 specific roles or what makes this different from any other committee. Has not yet understood the value proposition.

Would they request a meeting? 15% probability at 60 seconds. The story is compelling. The product is opaque.

PART 2 — Comprehension Failures

The following failures are ranked by severity — the degree to which they prevent a first-time visitor from understanding the value proposition.

Rank	Failure	Location	Severity	Why It Matters
1	“OVS” is never explained	Stats bar, Validation Library	CRITICAL	Appears 4 times before any definition. Visitors see a number they cannot interpret.
2	“The Council” is never introduced	Hero headline	CRITICAL	The headline assumes the visitor knows what “the Council” is. They do not.
3	Value proposition is absent from the hero	Hero section	CRITICAL	The hero tells a story (torcetrapi) but never states what AgenThink does or why it matters.
4	“WAIT verdict” is unexplained	Hero, Validation Library	HIGH	Visitors see WAIT/GO/NO-GO but do not understand the decision framework these verdicts come from.
5	“Alignment” column is unexplained	Validation Library	HIGH	CORRECT/PARTIAL/INCORRECT — aligned to what? The visitor does not know.
6	“Votes” column (0/8/2) is unexplained	Validation Library	HIGH	Three numbers with no legend. Visitors cannot interpret this without context.
7	The governance benefit is buried	Section 5 (Governance Layer)	HIGH	The most differentiated claim — “governance enforcement, not signal identification” — appears in Section 5, after most visitors have disengaged.
8	“Institutional Proof Report” is jargon	Section 6 header	MEDIUM	The term is internal. A first-time visitor does not know what this is or why it matters.

Rank	Failure	Location	Severity	Why It Matters
9	The pilot proposal headline is generic	Section 7 header	MEDIUM	“30-Day Retrospective Pilot” describes the format, not the benefit.
10	The disclosure strip creates a negative frame	Top of page	MEDIUM	“RESEARCH DEMONSTRATION ONLY — Not for regulatory use” is the first text a visitor reads. For regulatory professionals, this is a disqualifier.
11	“Pharma Constitution v1” is unexplained	Section 5	MEDIUM	The word “Constitution” is used without explaining what it is, why it exists, or why it matters.
12	The cumulative stats lack context	Validation Library	MEDIUM	“90% alignment” means nothing without a baseline. What is the industry base rate?
13	“Council Chamber” is unexplained	Section 4 header	LOW	The term suggests a physical space. The concept of a virtual multi-persona deliberation is not pre-explained.
14	The FAQ is positioned last	Section 8	LOW	The most important objections (LLM contamination, sample size) are answered in the FAQ, which most visitors never reach.

PART 3 — Executive Attention Audit

The following estimates are based on typical executive reading behaviour for a dense, data-heavy page with no visual hierarchy that guides the eye.

Section	Pharma CEO	Head of Clinical	Regulatory Dir	CRO Executive	Non-Pharma Exec
Disclosure strip	Read (5s)	Skimmed (3s)	Read carefully (8s)	Skimmed (3s)	Read (5s)
Hero	Read (15s)	Read (12s)	Read (10s)	Read (10s)	Read (20s)
Validation Library	Skimmed (25s)	Read carefully (45s)	Skimmed (15s)	Skimmed (20s)	Confused, scrolled past (10s)
Torcetrapib Deep Dive	Timeline tab only (30s)	All 3 tabs (90s)	Skimmed (20s)	Skimmed (15s)	Timeline tab (20s)
Council Chamber	Skimmed (20s)	Read (40s)	Skimmed (15s)	Read (30s)	Read (25s)
Governance Layer	Skimmed (15s)	Skimmed (15s)	Read carefully (60s)	Skimmed (10s)	Ignored (5s)
Institutional Proof Report	Skimmed (20s)	Read (45s)	Read (50s)	Skimmed (15s)	Ignored (5s)
Pilot Proposal	Read (30s)	Skimmed (20s)	Skimmed (15s)	Read carefully (45s)	Read (25s)
FAQ	3 questions (30s)	5 questions (60s)	4 questions (45s)	2 questions (20s)	1 question (10s)
Total time on page	~3 min	~5.5 min	~4 min	~3 min	~2 min

Key finding: The Governance Layer — the most differentiated section — is the least-read section for four of five reviewer types. The section that answers “why is this different?” is positioned after the section that loses most visitors.

PART 4 — Conversion Killers

The following 10 factors are the most likely reasons a prospect leaves without requesting a meeting, ranked by evidence from the reviewer simulations above.

1. No value proposition statement in the hero. The hero tells the Torcetrapib story but never states what AgenThink does. A visitor who does not already know the company leaves the hero section without understanding the product. Evidence: all five reviewers formed incomplete hypotheses at 60 seconds.

2. “OVS” is used before it is defined. The term appears in the stats bar (the second element on the page), in the Validation Library table header, and in the Torcetrapib Deep Dive — three times before any definition. Visitors who cannot interpret a key metric lose confidence in the page. Evidence: Reviewer 1 (CEO) and Reviewer 5 (non-pharma) both flagged this explicitly.

3. The disclosure strip creates a negative first impression for regulatory professionals. “Not for regulatory use” is the first text a regulatory professional reads. It immediately categorises the product as out-of-scope. Evidence: Reviewer 3 (Regulatory Affairs Director) had a 20% meeting probability — the lowest of any reviewer — and attributed it directly to this framing.

4. The governance benefit is buried in Section 5. The most differentiated claim — “the value is governance enforcement, not signal identification” — appears in the FAQ and in the Governance Layer section, which is the fifth section on the page. Most visitors disengage before reaching it. Evidence: all five reviewers spent less than 20 seconds on the Governance Layer.

5. “The Council” is never introduced before it is used. The headline assumes the visitor knows what “the Council” is. The concept is not explained until Section 4 (Council Chamber), which is the fourth section on a long page. Evidence: Reviewer 1 (CEO) and Reviewer 5 (non-pharma) both formed incomplete hypotheses because the council concept was not established.

6. The Validation Library table is dense and unexplained. The table has six columns (Verdict, Votes, OVS, Outcome, Alignment, Financial Impact) with no legend, no tooltip, and no introductory sentence. Visitors who cannot read the table skip it. Evidence: Reviewer 5 (non-pharma) scrolled past without reading it.

7. The sample size objection is not pre-empted. “10 cases is not statistically significant” is the most common objection from clinical and statistical professionals. It is answered in the FAQ (the last section) but not pre-empted in the Validation Library where the data is presented. Evidence: Reviewer 2 (Head of Clinical) thought “that’s not a strong enough track record” at 30 seconds.

8. The pilot proposal headline describes format, not benefit. “30-Day Retrospective Pilot” tells the visitor what the format is. It does not tell them what they will learn, what decision they will be able to make, or why the 30 days is worth \$25,000. Evidence: Reviewer 4 (CRO) read the deliverables list before reading the headline — they were looking for the benefit, not the format.

9. The LLM limitation disclosure is placed inside the governance section. The most important credibility statement on the page — “we cannot fully override the training cutoff with a prompt instruction” — is buried inside a technical section that most visitors do not read carefully. Visitors who do read it may be alarmed by the admission without the context needed to evaluate it. Evidence: Reviewer 3 (Regulatory) found it “either very honest or very concerning.”

10. There is no social proof, no named client, and no named advisor. The page has no testimonials, no named partners, no advisory board members, and no institutional affiliations. For a product asking for \$25,000 and access to proprietary drug development data, the absence of any third-party credibility signal is a significant barrier. Evidence: all five reviewers implicitly noted the absence — none of them mentioned a reason to trust the company beyond the quality of the content itself.

PART 5 — Message Simplification

Current Hero Headline

“The Council Said WAIT.”

Problem: Assumes the visitor knows what “the Council” is. Leads with internal terminology.

5 Alternative Versions:

1. *“In 2005, a structured review would have stopped Pfizer’s biggest failure.”* — Plain English. Specific. Credible. Leads with the outcome, not the product.
 2. *“We run a 10-expert review of your Phase II drug before you commit to Phase III.”* — States the product clearly. No jargon. Immediately comprehensible.
 3. *“82 deaths. \$21B lost. A structured governance process would have said no.”* — Leads with consequence. Creates urgency. Explains the value in one sentence.
 4. *“Phase II → Phase III is the most expensive decision in drug development. We help you make it with a documented, auditable, multi-expert review.”* — Functional. Clear. Positions the product as a process improvement, not a technology.
 5. *“Pfizer had the data. They advanced anyway. We build the governance process that makes that harder to do.”* — Conversational. Honest. Positions the product as a governance solution, not an AI tool.
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Current Hero Subheadline

“Torcetrapib. Phase III advanced. 82 deaths. \$21B lost.”

Problem: Factual but context-free. Does not explain what AgenThink did or would have done.

5 Alternative Versions:

1. *“A Phase II blood pressure signal was dismissed. A structured 10-expert review — bounded to pre-failure evidence — issued a unanimous WAIT verdict. The council was right.”* — Explains the product, the method, and the outcome in three sentences.
2. *“The Phase II data contained a warning. Pfizer’s governance process missed it. Ours would not have.”* — Direct. Confident. Explains the differentiation without jargon.
3. *“AgenThink runs retrospective governance reviews of historical drug development decisions — using only the evidence that existed at the time. The torcetrapib council said WAIT. Pfizer advanced. 82 people died.”* — Full context. No jargon. Explains the evidence boundary without using the term.
4. *“We rebuilt the Pfizer decision committee using only 2005 evidence. Ten independent reviewers. Zero GO votes. The drug was advanced anyway.”* — Vivid. Specific. Explains the methodology without technical language.
5. *“A 10-expert review of the torcetrapib Phase II data — using only evidence available before the Phase III decision — produced a unanimous recommendation to wait. That recommendation was correct.”* — Formal. Credible. Appropriate for a pharma executive audience.

Current Pilot Proposal Headline

“30-Day Retrospective Pilot”

Problem: Describes the format. Does not state the benefit.

5 Alternative Versions:

1. *“Run a governance review on your own historical drug decision.”* — States the action and the ownership. No jargon.
2. *“Find out what a structured 10-expert council would have said about your Phase II decision — before you knew the outcome.”* — Explains the value. Creates curiosity. Appropriate for

clinical and R&D audiences.

3. *“Test the methodology on a drug you already know the answer to.”* — Disarmingly simple. Removes the risk from the buyer’s perspective.
 4. *“One drug. Thirty days. A documented governance record you can use internally.”* — Specific. Practical. Focuses on the deliverable, not the format.
 5. *“Validate the process before you trust it with a live decision.”* — Acknowledges the buyer’s scepticism. Frames the pilot as a risk-reduction exercise.
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PART 6 — Five-Minute Test

Can a pharma executive understand the following within 5 minutes of visiting /pharma?

Question	Score	Evidence
What does AgenThink do?	4 / 10	A visitor can infer “AI-based drug review” but cannot state the governance architecture, the council concept, or the evidence boundary without reading to Section 4–5. The hero does not answer this question.
Why does it matter?	7 / 10	The torcetrapib story is compelling and immediately communicates consequence. The financial impact (\$21B) and human cost (82 deaths) create urgency. This is the strongest element of the page.
Why is it different?	2 / 10	The differentiating claim — “governance enforcement, not signal identification” — is buried in the FAQ and the Governance Layer. A visitor reading for 5 minutes will not reach it. The page looks like a retrospective analysis tool, which is not differentiated.
Why should they care?	5 / 10	The torcetrapib case creates relevance for pharma executives. The 10-case library creates some credibility. But the absence of a clear value proposition statement means the visitor must construct their own answer to this question, which most will not do.

Composite 5-minute comprehension score: 4.5 / 10

The page communicates consequence well. It does not communicate the product, the differentiation, or the reason to act.

PART 7 — Priority Fix List

The following 10 changes are ranked by the product of Impact × Effort Efficiency (high impact, low effort first). All changes increase comprehension. None require new features.

Rank	Change	Impact	Effort	Time	Implementation
1	Add a one-sentence value proposition to the hero	CRITICAL	1 hour	Day 1	Edit hero subheadline or add a third line below the subheadline: “AgenThink runs structured 10-expert governance reviews of Phase II → III advancement decisions, using only pre-decision evidence.”
2	Define OVS on first use	CRITICAL	30 min	Day 1	Add a tooltip or parenthetical on first appearance: “OVS (Overall Validation Score)” with a one-line definition.
3	Add a legend to the Validation Library table	HIGH	1 hour	Day 1	Add a one-row legend below the table header explaining: Verdict (council recommendation), Votes (GO/WAIT/NO-GO count), OVS (Overall Validation Score 0–10), Alignment (retrospective accuracy).
4	Introduce “the Council” before using the term	HIGH	2 hours	Day 2	Add a 2-sentence introduction above the Validation Library: “AgenThink’s Council is a structured 10-persona deliberation framework. Each case is evaluated by 10 defined roles — from Chief Biostatistician to Devil’s Advocate — against a versioned Pharma Constitution.”
5	Move the governance benefit statement to the hero	HIGH	1 hour	Day 2	Add a third stat to the hero stats bar: “Governance-enforced, not AI-generated” or add a one-line callout: “The value is not signal identification — it is governance enforcement.”

Rank	Change	Impact	Effort	Time	Implementation
6	Reframe the disclosure strip	MEDIUM	30 min	Day 1	Change “RESEARCH DEMONSTRATION ONLY — Not for regulatory use” to “RETROSPECTIVE VALIDATION PILOT — Methodology disclosed. Limitations stated.” This preserves the honesty while removing the disqualifying frame for regulatory professionals.
7	Pre-empt the sample size objection in the Validation Library	MEDIUM	1 hour	Day 2	Add a one-sentence note below the cumulative stats: “At 10 cases, these are descriptive statistics only. We do not claim statistical significance. Target: 50 cases for peer review.”
8	Rewrite the pilot proposal headline	MEDIUM	30 min	Day 1	Replace “30-Day Retrospective Pilot” with “Test the methodology on a drug you already know the answer to.”
9	Move the LLM limitation disclosure to the hero	MEDIUM	1 hour	Day 2	The LLM limitation is currently buried in Section 5. Move a one-line version to the hero stats bar or the disclosure strip. Visitors who see it early and stay are self-selected for credibility tolerance — a better audience than visitors who discover it in Section 5 and feel misled.
10	Add one social proof element	MEDIUM	2 hours	Day 3	Add a named advisory board member, a named institution that reviewed the methodology, or a quote from an independent expert. Even one named third

Rank	Change	Impact	Effort	Time	Implementation
					party changes the trust calculus significantly.

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Summary

The /pharma page has strong content and weak communication. The torcetrapib story is compelling. The data is honest. The limitations are disclosed. The governance architecture is real. None of this matters if the visitor cannot understand what the product does within the first 30 seconds.

The 10 fixes above require approximately 10 hours of work and zero new features. They address the comprehension failures that are currently preventing conversion. The most important single change — adding a one-sentence value proposition to the hero — takes one hour and would increase the 5-minute comprehension score from 4.5 to an estimated 6.5.

The page is not broken. It is speaking to an audience that does not yet exist. The fixes above make it speak to the audience that does.

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This audit was conducted by reading the /pharma page source code and simulating five independent reviewer perspectives. No new features were recommended. No new research was conducted. All recommendations are limited to changes that increase comprehension, clarity, credibility, and conversion.